

ARCP Evidence Checklist

Quality improvement evidence for AIM trainees — what your portfolio needs, by training year.

THE HEADLINE

ST4–ST6 QI is **one** route to your annual clinical-governance evidence — alongside audit, Datix review, complaints response or a Coroner's referral. You don't have to lead a full project every year, but keep building a track record.

ST7 Bring **one** QIP to completion: a full project plan and report, assessed with the Quality Improvement Project Assessment Tool (QIPAT). Starting that project well before ST7 makes this far less last-minute.

YEAR BY YEAR

STAGE	WHAT'S REQUIRED	TOOLKIT RESOURCE TO USE
ST4	Evidence of engagement with QI, audit, a Datix review, a complaints response, or a Coroner's referral — QI is one acceptable option, not the only one.	Contribute to a ward project; keep your PDSA worksheets and run-chart entries as your evidence.
ST5	Same annual engagement requirement — a good year to move from contributing to co-leading.	PDSA Cycle Worksheet, Run Chart / SPC Worksheet.
ST6	Same requirement — start pitching your own project ahead of ST7.	SMART Aim Statement Builder, QIP Roadmap (Stages 1–3).
ST7	One full QIP: project plan and report, assessed with QIPAT.	Full SQUIRE 2.0 write-up (Reporting Guide + Presentation Template); QIPAT sign-off from your supervisor.

EVIDENCE TO KEEP, MAPPED TO SQUIRE 2.0

EVIDENCE	WHAT IT SHOWS THE PANEL	SQUIRE 2.0 ITEM
SMART aim statement	A clearly defined, measurable problem — not just “things were bad.”	<i>Item 6, Specific Aims</i>
PDSA cycle worksheet, each cycle you run	Iterative testing and real learning from data, not one big-bang change.	<i>Item 13a, Evolution of the Intervention</i>
Run Chart / SPC Worksheet, kept updated	You understand your data over time, not just a before-and-after snapshot.	<i>Item 11, Analysis; Item 13b, Process & Outcome Data</i>
QIPAT form, completed with your supervisor	Formal, supervisor-assessed sign-off — required for your ST7 project.	<i>Curriculum requirement at ST7</i>
Completed SQUIRE 2.0 write-up	A finished, structured report, ready for your portfolio or a presentation.	<i>All 18 items</i>
Presentation or dissemination — local QI meeting, SAM conference, trust newsletter	Evidence you shared what you found, not just that you did the work.	<i>Item 15, Interpretation</i>

A FEW PRACTICAL NOTES

- **Protected time** isn't set out separately for QI in the AIM curriculum. In practice it tends to come out of your general specialist-skills time — ask your TPD or educational supervisor how your programme handles this locally.
- **QIPAT is only formally required once**, for your ST7 project — but SAM's own training guidance suggests aiming for roughly one QIPAT-assessed project a year where you can. That's a track record, not a rule.
- **Don't leave it to the week before ARCP.** A write-up takes real time to do properly, and your supervisor needs time to complete the QIPAT assessment with you — start that conversation early.
- **Simply attending QI teaching isn't evidence.** The panel is looking for your own engagement — ideally moving toward a leadership or co-leadership role by ST7.

Sources: Acute Internal Medicine 2022 ARCP Decision Aid, Joint Royal Colleges of Physicians Training Board / the Federation of the Royal Colleges of Physicians of the UK (thefederation.uk/sites/default/files/AIM%202022%20ARCP%20Decision%20Aid%20FINAL.pdf); Society for Acute Medicine, "Training Information for AIM Higher Specialty Trainees" (acutemedicine.org.uk/training-in-acute-medicine).